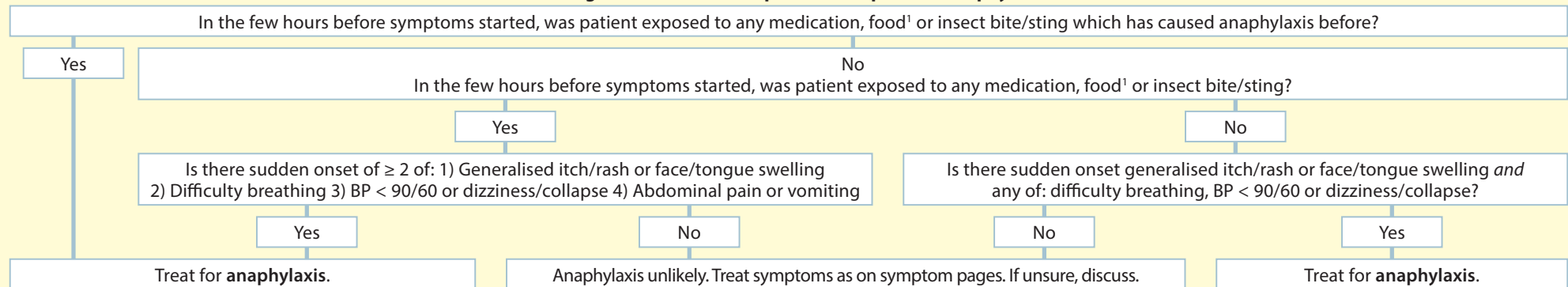


ANAPHYLAXIS

Give urgent attention to the patient with possible anaphylaxis:



Manage anaphylaxis and refer urgently:

- Give immediately **adrenaline**² 0.5mL (1:1000 solution) IM into mid outer thigh. Repeat every 5 minutes if needed.
- Raise legs and give 100% face mask oxygen.
- Give **sodium chloride 0.9%** 1-2L IV rapidly regardless of BP. Then, if BP < 90/60, also give **sodium chloride 0.9%** 500mL IV rapidly, repeat until systolic BP > 90. Continue 1L 6 hourly. Stop if breathing worsens.
- If persistent wheeze or difficulty breathing despite adrenaline², also give 1mL **salbutamol 0.5%** solution and 2mL **ipratropium bromide** solution in 4mL **sodium chloride 0.9%** via nebuliser every 20 minutes for 3 doses. If needed, assess and further manage airway ↗ 14.
- Give **hydrocortisone** 200mg IM/slow IV immediately *and* **promethazine** 50mg IM/slow IV.

Assess the patient with previous anaphylaxis

Assess	When to assess	Note
Trigger	At diagnosis	Ensure a specialist has reviewed the patient with anaphylaxis to confirm trigger/s. Common triggers include medications, food ¹ and insect bites/stings.
Other allergy	At diagnosis	• If recurrent cough, wheeze, tight chest or difficulty breathing, exclude asthma ↗ 123. If known asthma, give routine asthma care ↗ 125. • If patches of dry, scaly, itchy skin on wrists, ankles, inside elbows or behind knees, eczema likely ↗ 69. • If itchy, red, raised wheals that appear suddenly and usually disappear within 24 hours, urticaria likely ↗ 69. • If recurrent sneezing or itchy/runny/blocked nose most days for > 4 weeks, allergic rhinitis likely ↗ 34. • If both eyes watery and itchy, allergic conjunctivitis likely ↗ 31.

Advise the patient with previous anaphylaxis

- Advise to avoid identified trigger/s and if trigger is a medication, to always inform health worker.
- Ensure patient has a plan in case of anaphylaxis: ambulance telephone number, nearest hospital and reliable transport plan.
- If adrenaline² auto-injector device (like EpiPen®) prescribed, ensure patient knows when and how to use it:
 - If exposed to trigger, use immediately if any of: itch/rash, face/tongue swelling, itchy/tight throat, cough, wheeze, difficulty breathing, dizziness/collapse, abdominal pain or vomiting. After use, immediately phone for ambulance.
 - Advise to read instructions found in packaging.
- Arrange a MedicAlert® bracelet ↗ 178 and advise patient to always wear it.

¹Common foods causing anaphylaxis include peanuts, tree nuts, egg, milk and fish. ²Adrenaline is also known as epinephrine.